Harborview Orthopedic Center — Joint Reconstruction

OPERATIVE REPORT — TOTAL JOINT ARTHROPLASTY

Patient: Gerald S. Burnett **DOB: 01/08/1956** **MRN: 3094718**

Date: November 12, 2024 Provider: Dr. Patricia Lim, MD — Orthopedic Surgery

OPERATIVE SUMMARY

Pre-op Diagnosis:	Severe tricompartmental osteoarthritis, right knee (M17.11)	**Procedure:**	Right total knee arthroplasty (TKA) with patella resurfacing
Implant System:	Zimmer Biomet Persona — Cemented PS (Posterior Stabilized)	**Femoral Component:**	Size 6, standard
Tibial Component:	Size 4, 10mm articular insert (cruciate-stabilized)	**Patellar Component:**	35mm dome, all-polyethylene
Tourniquet Time:	68 minutes (inflated at 275 mmHg)	**EBL:**	< 200 mL (TXA 1g IV given pre-incision and 3 hours post)
Complications:	None		

OPERATIVE TECHNIQUE

The patient was positioned supine on the operating table with a lateral post at the thigh. Spinal anesthesia was administered with adductor canal nerve block under ultrasound guidance (0.5% ropivacaine 30 mL). A tourniquet was applied to the right thigh.

A midline skin incision was made from 10cm proximal to the superior pole of the patella to the tibial tubercle. A medial parapatellar arthrotomy was performed, with medial release of the pes anserinus insertion and medial collateral ligament as needed to correct varus deformity. The patella was everted and the joint was exposed. Synovectomy was performed. The cruciate ligaments were identified — the ACL was attenuated; the PCL was intact and ultimately sacrificed to accommodate the PS implant.

Bony resections were performed using standard jigs per manufacturer specifications: distal femoral resection at 9°valgus, posterior condylar resection, anterior chamfer cuts, and proximal tibial resection perpendicular to mechanical axis. Patellar resection performed to 10mm residual bone thickness, measured at 35mm diameter.

Trial components were placed and confirmed optimal sizing via fluoroscopy. Alignment: full mechanical axis correction from 8° varus to neutral confirmed. Gap balancing: extension and flexion gaps equal at 10mm. Range of motion with trials: 0-128°.

Cement mixing per manufacturer protocol. Final components were press-fit then cement fixed. Excess cement removed under tourniquet. Tourniquet released. Hemostasis achieved. Wound irrigated with 3L saline. Intraarticular drain placed (removed POD 1). Closure: retinaculum with 0 Vicryl interrupted, subcutaneous with 2-0 Vicryl, skin with staples. Compressive dressing applied.

POSTOPERATIVE PLAN

Weight-bearing as tolerated with front-wheeled walker beginning POD 1. Physical therapy to begin POD 1 with gait training and ROM exercises. DVT prophylaxis: enoxaparin 40mg SC daily x 10 days (consider extended 28 days given BMI). Cryotherapy PRN for swelling. VTE risk assessment: moderate-high (TJA + prior immobility); mechanical and pharmacological prophylaxis ordered.

Discharge expected POD 2-3 to home with home PT if independent with walker and able to negotiate stairs, or to skilled nursing facility if additional support needed. Follow up with Dr. Lim at 2 weeks for staple removal, 6 weeks for functional assessment, and 1 year with weight-bearing knee radiographs.